

- (b) If the reported sites are in different anatomical systems, or if there is no residual category for the group or anatomical system, code to the residual category for the disease or condition specified.

Example 2

- 1 (a) Cancer of adrenal gland or kidney
 - (b)
 - (c)
 - (d)
- 2

Code as [2D42](#) *Malignant neoplasms of ill-defined sites*, since adrenal gland and kidney are in different anatomical systems.

2.21.1.3 One site or system, either one condition or another condition

- (a) If the reported conditions are classifiable to different subcategories, and ICD provides a group or category for the disease in general, code to the residual category of this group/category.

Example

1

- 1 (a) Sigmoid volvulus [DB30.1](#) or adhesions of large intestine with obstruction [DB30.2](#)
 - (b)
 - (c)
 - (d)
- 2

Since both sigmoid volvulus ([DB30.1](#)) and adhesion of large intestine with obstruction ([DB30.2](#)) are in the same group, code to the residual category [DB30.Z](#) *Obstruction of large intestine, unspecified*

Example

2

- 1 (a) Dissection of cerebral arteries [8B22.0](#) or cerebral infarction [8B11.5Z](#)
 - (b)
 - (c)
 - (d)
- 2

Since dissection of the cerebral arteries ([8B22.0](#)) and cerebral infarction ([8B11.5Z](#)) are in the same group, code to the residual category [8B2Z](#) *Cerebrovascular diseases, unspecified*.

- (b) If there is no group or category for the disease in general, code to the residual category of the disease of the anatomical site/system common to the reported conditions.

Example 3

- 1 (a) Tuberculosis or cancer of lung
- (b)
- (c)
- (d)
- 2

Code as [CB40.Y](#) *Other specified diseases of the respiratory system*. Both conditions involve the lung.

Example 4

- 1 (a) Stroke or heart attack
- (b)
- (c)
- (d)
- 2

Code as [BE2Z](#) *Diseases of the circulatory system, unspecified*. Although stroke is classified to the nervous system chapter, both conditions are diseases of the circulatory system.

2.21.1.4 Either one condition or another, different anatomical systems

When different diseases of different anatomical systems are reported as 'either ... or', code to [MG6Y](#) *Other specified clinical findings in specimens from other specified organs, systems or tissues*.

Example 1

- 1 (a) Gallbladder colic or coronary thrombosis
- (b)
- (c)
- (d)
- 2

Code as [MG6Y](#) *Other specified clinical findings in specimens from other specified organs, systems or tissues*.

2.21.1.5 Either disease or injury

When death is reported as due to either a disease or an injury, code to [MH14](#) *Other ill-defined or unspecified causes of mortality*.

Example 1

- 1 (a) Coronary occlusion or war injuries
 - (b)
 - (c)
 - (d)
- 2

Code as [MH14](#) *Other ill-defined or unspecified causes of mortality*.

2.21.2 Effect of connecting terms

When the certifier uses a connecting term, the codes assigned must be arranged to reflect the certifier intention. There are two types of connecting terms: those implying a causal relationship, and those not implying a causal relationship between reported causes of death.

2.21.2.1 Connecting terms implying a causal relationship

A causal relationship can be expressed in two ways: 'due to' written or implied by a similar term; or 'resulting in' written or implied by a similar term. This applies to other connecting terms or signs that indicate a 'due to' relationship, such as 'caused by', 'because of', or similar.

'Due to' written or implied by a similar term

When one cause is certified with a connecting term implying it is due to another cause, enter the code for the first cause on the line where reported and the code for the other cause on the next lower line. Code any causes reported on the remaining lines in Part 1 on the next lower lines.

Example 1

- 1 (a) Heart failure due to ischaemic heart disease
 - (b) Diabetes
 - (c)
 - (d)
- 2

Heart failure is the first cause on line (a), so code it to line (a). It is reported as due to ischaemic heart disease, so code ischaemic heart disease to line (b). Move diabetes, which is written on line (b), to line (c).

Example 2

- 1 (a) Heart failure due to hepatocellular carcinoma
- (b) Ischaemic heart disease
- (c) Diabetes
- (d)

2

Heart failure is the first cause on line 1(a), so code it to line (a). It is reported as due to hepatocellular carcinoma, so code hepatocellular carcinoma to line 1(b). Move ischaemic heart disease, which is reported on line 1(b), to line 1(c). Also move diabetes, which is reported on line 1(c), to line 1(d).

'Resulting in' written or implied by a similar term

When one cause is certified with a connecting term implying it resulted in another cause, enter the code for the cause following the connecting term on the line where reported, and the code for the cause preceding the connecting term on the next lower line. Code any causes reported on the remaining lines in Part 1 on the next lower lines.

Example 1

- 1 (a) Ischaemic heart disease resulting in heart failure
- (b) Diabetes
- (c)
- (d)

2

Code heart failure, which follows the connecting term 'resulting in', on line (a). Code ischaemic heart disease, which is reported before the connecting term, is moved to line (b). Move diabetes, reported on line (b), one line down and code it on line (c).

Example 2

- 1 (a) Hepatocellular carcinoma causing heart failure
- (b) Ischaemic heart disease
- (c) Diabetes
- (d)

2

Code heart failure reported after the connecting term 'causing', on line 1(a). Code hepatocellular carcinoma, which is reported before the connecting term, on line 1(b). Move ischaemic heart disease, reported on line 1(b), to line 1(c), and move diabetes, which is reported on line 1(c), to line 1(d). This applies to other connecting terms or signs that indicate a 'resulting in' relationship, such as 'causing', 'leading to', 'developing into', and similar.

2.21.2.2 Connecting terms not implying a causal relationship

'And' written or implied by a similar term first or last on a line

The connecting term 'and' does not imply a causal relationship, but it indicates that the terms before and after it should be counted. Therefore, when a line ends with 'and', code the cause(s) mentioned on the line immediately below for this line, so that the coding reflects the enumeration implied by the connecting term. Similarly, when a line starts with

'and', consider this as a continuation of an enumeration starting on the line above, and code the cause or causes on that line last on the line above. Code any causes reported on the remaining lines in Part 1 where reported. This applies to other connecting terms or signs that indicate an enumeration but do not imply a causal relationship, such as 'also', 'plus', 'besides', 'in addition', '+' or comma.

Example 1

- 1 (a) Heart failure and
(b) Ischaemic heart disease
(c) Diabetes
(d)
- 2

Line 1(a) ends with 'and', so consider 'ischaemic heart disease', reported on line (b) as a part of the enumeration 'heart failure and ischaemic heart disease'. Code accordingly and place the codes for both heart failure and ischaemic heart disease on line 1(a). Code diabetes on line (c).

Example 2

- 1 (a) Heart failure
(b) Ischaemic heart disease
(c) and diabetes
(d)
- 2

Line 1(c) starts with 'and'. Consider diabetes, reported on line (c), as a part of the enumeration 'ischaemic heart disease and diabetes'. Code accordingly, and place the codes for both ischaemic heart disease and diabetes on line 1(b).

'And' written or implied by a similar term but not first or last on a line

If a connecting term that does not imply a causal relationship is written on a line but not first or last, then treat it as a comma. Do not reformat the text and do not move any part of the causes reported to another line.

Diagnostic terms that do not stop at the end of the line

If a diagnostic term starts on one line in Part 1 and continues on the next line, code as if the entire diagnostic term had been written on the line where the diagnostic term starts. Code any causes reported on the remaining lines in Part 1 where reported.

Example 1

- 1 (a) Ischaemic
(b) Heart disease
(c) Diabetes type 2
(d)
- 2

'Ischaemic heart disease' is a diagnostic term reported on two lines. Code as if the complete term had been written on line (a). Code diabetes where it is reported, on line (c).

Example 2

- 1 (a) Pneumonia
- (b) Chronic kidney disease
- (c) Diabetes type 2
- (d)

2

'Chronic kidney disease' is a diagnostic term reported on two lines. Reformat the certificate and code the complete term 'chronic kidney disease' on line (b). Also code diabetes on line (b), since it continues the line where 'chronic kidney' has been written.

2.21.3 Duration of conditions

2.21.3.1 Single duration stated for multiple conditions

When more than one condition is reported in the same line with only one stated duration, consider that each condition reported had the same duration.

2.21.3.2 Modifying temporality of conditions by stated duration

Duration should not usually be used to qualify a condition as

- Acute
- Chronic
- Early onset
- Late onset

unless the Indexed Term provides specific duration criteria (e.g. [BA50](#) old myocardial infarction as an example) or it is otherwise instructed in the reference guide (e.g. Section [2.21.8.1 Acute or chronic rheumatic heart diseases](#)). Note that the Description in the classification is not to be used for coding (Section [3.4](#)).

2.21.4 'Code also' instructions in mortality use case

Generally, the 'code also' instruction (see also Section [2.7.2.1 'Code also' and 'Use additional code, if desired' instructions](#)) is not used in multiple cause coding since the information on aetiology is provided as a stand-alone expression separately on the death certificate and will be coded on its own, or is not provided at all.

Apply the 'Code also' instruction when both information on the manifestation and the aetiology appear in a single diagnostic term reported by the certifier, and information on the aetiology is not reported separately. Whenever applying the 'code also' instruction, put the code for the aetiology at the beginning of the cluster and add the code for the manifestation.

Example 1: Heart failure

[BD10-BD1Z](#) *Heart failure* has an instruction to 'Code also' the causing condition. However, in the diagnostic term reported by the certifier no information is given on such causing condition. Do not apply the 'Code also' instruction.

Example 2: Type 1 diabetic acidosis

[[5A22 Diabetic acidosis unspecified](#)] has an instruction to 'Code also' the causing condition. The causing condition is reported, which in this case is [5A10 Type 1 diabetes mellitus](#). The aetiological condition Type 1 diabetes mellitus is considered the causing condition for primary tabulation and is coded first ([5A10 Type 1 diabetes mellitus](#)/[[5A22 Diabetic acidosis unspecified](#)]).

Example 3: Salmonella Sepsis

Salmonella sepsis is an index term of [1G40 Sepsis without septic shock](#) which has an instruction to 'Code also' the causing condition supplemented by a coding note to code the type of infection first. The type of infection in this case is [1A09 Infections due to other Salmonella](#) and is coded first ([1A09/1G40](#)).

2.21.5 Malignant neoplasms

To assign the correct multiple cause code for a neoplasm, there are two concepts to consider: whether the reported neoplasms is primary or secondary, and its behaviour.

The primary site is the anatomical location where the neoplasm originated. A malignant neoplasm may spread to other parts of the body, and these sites are referred to as a secondary or metastasis.

The behaviour (malignant, in situ, benign, uncertain or unknown) of a neoplasm is the way it spreads within the body:

- Malignant - the neoplasm invades surrounding tissue or disseminates from its point of origin and begins to grow at another site
- In situ - the neoplasm is malignant but still fully confined to the tissue in which it originated
- Benign - the neoplasm grows in the place of origin without the potential for spread
- Uncertain behaviour - A neoplasm displaying morphologic, phenotypic, or genotypic characteristics that are clearly not benign but do not permit the establishment of a definitive diagnosis of malignancy
- Unknown behaviour - it is unknown whether the neoplasm is benign or malignant.

The broad structure of Chapter 02 'Neoplasms' given below can be understood from these concepts as follows:

- Neoplasms of brain or central nervous system ([2A00-2A0Z](#))
- Neoplasms of haematopoietic or lymphoid tissues ([2A20-2B3Z](#))
- Neoplasms, except of lymphoid, haematopoietic, central nervous system or related tissues ([2B50-2F9Z](#))

Neoplasms originating in the brain or central nervous systems, classified to [2A00-2A0Z](#), are further classified by site, histopathology or behaviour.

Neoplasms originating in the haematopoietic or lymphoid tissues, classified to [2A20-2B3Z](#), are further classified by histopathology.

In other words, [2A00-2A0Z](#) or [2A20-2B3Z](#) includes primary neoplasms, which could be either malignant or benign.

Neoplasms originating in other sites are classified to ([2B50]-[2F9Z](#)) and this group is further classified by behaviour (i.e. malignant, in situ, benign, uncertain, or unknown), site or histopathology. Note that neoplasms originating in other sites and disseminating into the brain, central nervous system, lymphoid, haematopoietic tissues, are included here under the block of malignant neoplasm metastases.

In mortality coding, it is most important to determine the primary site of the neoplasm. When the death certificate is ambiguous as to the primary site, every effort should be made to obtain clarification from the certifier. The instructions that follow should be applied only when clarification cannot be obtained.

In the examples in this section, ICD codes are provided to the right of the death certificate. These codes represent the multiple cause codes assigned to each entry. These multiple cause codes could be different from a code assigned when the given diagnostic entry was reported alone on the certificate (direct coding). In such cases, the code for direct coding is given in curly brackets '{ }' next to the diagnostic expression. The explanation for each example describes that the codes in brackets will be modified by other information on the certificate (application of multiple cause coding) and to code to the multiple cause code indicated to the right.

Using the coding tool for neoplasms

Using the ICD coding tool, search for the term reported on the certificate to describe the neoplasm. If both histopathology and site are stated, enter both into the coding tool. If the histopathology is incompatible with the stated site of the neoplasm (i.e. the neoplasm cannot be primary of the stated site according to textbooks and other reference literature), then assign a code for a neoplasm of unspecified site for the histopathology indicated. If the histopathology is not stated, code by site and behaviour, if reported.

Do not assign [2D43](#) *Malignant neoplasms of independent, multiple primary sites* when multiple neoplasms are mentioned. Code each malignant neoplasms by the coding tool, and select the underlying cause by applying the selection and modification rules in the normal way.

2.21.5.1 Behaviour: malignant, in situ, benign, uncertain or unknown behaviour

To assign the correct multiple cause code for a neoplasm, first determine behaviour (malignant, in situ, benign, uncertain or unknown) for each of the neoplasms reported on the death certificate. For malignant neoplasms, also determine whether to code them as primary or secondary. To that end, apply the instructions that follow.

The term itself indicates behaviour

First, use the coding tool to assign a code for terms used to describe the neoplasms. A specific histopathology or term may be assigned to a certain group of behaviour.

For neoplasms coded to the following categories, presume it behaviour is malignant, unless otherwise specified, and go to Section [2.21.5.2 Malignant neoplasms: primary or secondary?](#):

- [2A00-2A0Z](#) Neoplasms of central nervous system or related structures
- [2A20-2B3Z](#) Neoplasms of haematopoietic or lymphoid tissues
- [2A02.3](#) Benign neoplasm of cranial nerves

Note that the behaviour given by the coding tool for a specific term may change by other information on the certificate. Follow the instructions in this Section to decide the correct behaviour in multiple cause coding.

Other information on the certificate indicates behaviour

If the term used to describe the neoplasm does not indicate a specific behaviour, then look for other information on the certificate indicating behaviour. Code a neoplasm of unspecified behaviour, a neoplasm described as 'in situ', or a growth that is not coded to Chapter 02 (for example, certain polyps), as malignant if any of the conditions below apply:

- it is reported as the cause of secondary spread (terms such as infiltration, metastases, secondaries or similar) or of (malignant) cachexia, or;
- it is reported immediately beside a mention of secondary spread, or;
- all other neoplasms reported are specified as secondary spread, or;
- there is no mention of another neoplasm on the same part of the certificate, but there are other indications of malignancy reported anywhere on the certificate (for example, carcinosis, malignant cachexia, malignant transformation), or;
- it is reported as due to a malignant neoplasm.

If a neoplasm is coded to the Chapter 02 category for benign neoplasm but is reported as the cause of metastases or infiltration, check for a code for a malignant variety in the coding tool and in the tabular list. If found, code it as malignant. If there is no code for a malignant variety, first try to obtain clarification from the certifier. If no further information is available, then accept the statement on the certificate and code the neoplasm as benign.

If other information on the certificate indicates that a neoplasms is malignant, go to Section [2.21.5.2 Malignant neoplasms: primary or secondary?](#) to assign the correct code.

If there is no other information on the certificate that indicates behaviour, code as unknown behaviour.

Example 1

- 1 (a) Liver metastases [2D80.0](#)
- (b) Colon tumour {[2F90.0](#)} [2B90.Z](#)
- (c)
- 2

The colon tumour is reported as the cause of liver metastases so code as Malignant neoplasms of colon, unspecified ([2B90.Z](#)).

Example 2

- 1 (a) Cancer cachexia [MG20.0](#)
(b) Colon tumour {[2F90.0](#)} [2B90.Z](#)
(c)
2

The colon tumour is reported as the cause of malignant cachexia so code as Malignant neoplasms of colon, unspecified ([2B90.Z](#)).

Example 3

- 1 (a) Liver and lung metastases [2D80.0](#), [2D70](#)
(b) Respiratory failure [CB41.2Z](#)
(c) Colon tumour {[2F90.0](#)} [2B90.Z](#)
2

Both metastases and respiratory failure can be due to a colon tumour. According to the instructions on how to interpret causal relationships in Part 1 (Step SP3) this means that two valid causal relationships are reported on this certificate, 1) liver and lung metastases due to colon tumour, and 2) respiratory failure due to colon tumour. These relationships are valid even if liver and lung metastases cannot be due to respiratory failure. The colon tumour is reported as the cause of secondary spread so code as Malignant neoplasms of colon, unspecified ([2B90.Z](#)).

Example 4

- 1 (a) Colon tumour {[2F90.0](#)} with liver metastases [2B90.Z](#), [2D80.0](#)
(b)
(c)
2

The colon tumour is reported on the same line as and next to liver metastases so code as Malignant neoplasms of colon, unspecified ([2B90.Z](#)).

Example

5

- 1 (a) Breast tumour, generalized atherosclerosis, [2F95](#), [BD40.Z](#),
colon cancer with liver metastases [2B90.Z](#), [2D80.0](#)
(b)
(c)
2

The breast tumour is reported on the same line as but not next to secondary spread. Do not consider it malignant. Code as Neoplasms of unknown behaviour of breast ([2F95](#)).

Example 6

- 1 (a) Respiratory failure [CB41.2Z](#)
(b) Colon tumour {[2F90.0](#)} [2B90.Z](#)
(c)
2 Liver and lung metastases [2D80.0](#), [2D70](#)

All other neoplasms are specified as secondary spread so code colon tumour as Malignant neoplasms of colon, unspecified ([2B90.Z](#)).

Example 7

- 1 (a) Colon tumour {[2F90.0](#)} [2B90.Z](#)
(b)
(c)
- 2 Cancer cachexia [MG20.0](#)

There is no mention of another neoplasm but cancer cachexia, another indication of malignancy, is reported in Part 2 so code colon tumour as Malignant neoplasms of colon, unspecified ([2B90.Z](#)).

Example 8

- 1 (a) Bladder tumour [2F98](#)
(b) Lung tumour [2F91.1](#)
(c)
- 2

There is no information on the certificate that indicates behaviour, code bladder tumour as Neoplasms of unknown behaviour of urinary organs ([2F98](#)) and lung tumour as Neoplasms of unknown behaviour of trachea, bronchus or lung ([2F91.1](#)).

2.21.5.2 Malignant neoplasms: primary or secondary?

If a neoplasm is coded as malignant or its behaviour is presumed to be malignant by Section [2.21.5.1 Behaviour: malignant, in situ, benign, uncertain or unknown behaviour](#) above, next decide whether it is primary or secondary.

Sometimes malignant neoplasms are described as 'metastatic', which might refer either to a primary malignant neoplasm that has metastasized to another site, or to secondary malignant neoplasms originating somewhere else. For instructions on how to code neoplasms described as 'metastatic', see Section [2.21.5.6 'Metastatic' cancer](#).

Common sites of metastases

When choosing between codes for primary and secondary malignant neoplasms, refer to the following list of common sites of metastases:

- bone ([2B5Z](#))
- brain ([2A00.5](#))
- diaphragm ([2B5K](#) & [XA2JL0](#))
- ill-defined site ([2D42](#))
- liver ([2C12.0Z](#))
- lung ([2C25.Z](#) & [XA57M6](#))
- lymph nodes ([2D60-2D6Z](#))
- mediastinum ([2C28.Z](#) & [XA7WA2](#))
- meninges ([2A01.2](#))
- peritoneum ([2C51.Z](#))
- pleura ([2C28.Z](#) & [XA5TT2](#))
- retroperitoneum ([2C50.Z](#))
- spinal cord ([2A02.2](#))
- omentum ([2C52.Z](#))

Note: these sites should be considered common sites of metastases, unless a specific histopathology is described.

See below for further instructions on how to code these neoplasms.

Malignant neoplasm reported as primary

Code a malignant neoplasm specified as primary if it is specified as 'primary', 'primary in', 'originating in', or other similar terms.

Other indication of primary malignant neoplasm

Code a malignant neoplasm as primary, although not described as primary by the certifier, if:

- all other malignant neoplasms on the certificate are specified as secondary or as metastases. This applies whether the site not specified as secondary or as metastasis is on the list of common sites of metastases.
- it is in the code range [2A20-2B3Z](#) *Neoplasms of haematopoietic or lymphoid tissues*: Code all malignant neoplasms of haematopoietic or lymphoid tissues as primary, unless the certifier specifies them as secondary;
 - *note*: A primary neoplasm of haematopoietic and lymphoid tissues may occur simultaneously with another primary neoplasm in the same code range.
- the site is not on the list of common sites of metastases.

If the site is on the list of common sites of metastases, code the malignant neoplasm as primary if:

- the histopathology indicates that it is primary of the reported site;
- it is described as caused by a known risk factor for malignant neoplasms of the stated site (To determine if the condition reported as causing the neoplasm is a

known risk factor, check if it is mentioned as a risk factor of the site involved in textbooks or other reliable sources);

- it is the only malignant neoplasm mentioned on the death certificate, and it is not specified as 'metastatic':
 - *exception*: code malignant neoplasm of lymph nodes as secondary, even if it is the only reported neoplasm on the certificate, unless it is specified as primary;
 - *note*: if the only malignant neoplasm reported on the certificate is malignant neoplasm of liver, and it is not specified as either primary or secondary, then code it as primary;
- it is malignant neoplasm of lung, and all other malignant neoplasms mentioned on the certificate are on the list of common sites of metastases;
 - *exception*: code lung as secondary if another malignant neoplasm is reported in the same part of the certificate (Part 1 or Part 2) and this other malignant neoplasm is coded as a primary malignant neoplasm;
- it is malignant neoplasm of lung specified as bronchogenic or of bronchus.

Code a neoplasm of behaviour other than malignant as primary malignant if it is reported as causing secondary or metastatic spread. See '**Other information on the certificate indicates behaviour**' above.

- *exception*: If durations are stated, the secondary neoplasms must not have a longer duration than the presumed primary malignant neoplasm;
- *exception*: If morphologies are stated, the secondary and presumed primary malignant neoplasms must have the same histopathology.

Do not code a neoplasm of behaviour other than malignant as primary malignant if it is reported as the cause of another neoplasm that would not be coded as malignant. Do not assume malignancy or metastatic spread. Code both neoplasms according to the coding tool.

Example 1

- 1 (a) Brain metastasis [2D50](#)
(b) Lung tumour [2C25.Z](#)
(c)
(d)
- 2

The lung tumour caused brain metastasis so is coded as malignant. The other malignant neoplasms on the certificate is described as metastasis ([2D50](#)) so code the lung tumour as primary at [2C25.Z](#)].

Example 2

- 1 (a) Cancer of pancreas [2C10.Z](#)
(b) Cancer of stomach [2B72.Z](#)
(c)
(d)

2

Pancreas and stomach are not on the list of common sites of metastases so code both as primary at Malignant neoplasm of pancreas, unspecified ([2C10.Z](#)) and Malignant neoplasms of stomach, unspecified ([2B72.Z](#)).

Example 3

- 1 (a) Cancer of liver and lung {[2C25.Z](#)} [2C12.02](#) [2D70](#)
(b) Chronic hepatitis [DB97.2](#)
(c)
(d)

2

Chronic hepatitis increases the risk of primary liver cancer so code the liver cancer as primary at Hepatocellular carcinoma of liver ([2C12.02](#)). Code the lung cancer as secondary at Malignant neoplasm metastasis in lung ([2D70](#)), because the other malignant neoplasm reported in the same part of the certificate is coded as primary.

Example 4

- 1 (a) Kidney cancer and lung cancer {[2C25.Z](#)} [[2C90](#)], [2D70](#)
(b)
(c)
(d)

2

Code the kidney cancer as primary ([2C90.Z](#)) since it is not on the list of common sites of metastases. Code lung cancer as secondary at Malignant neoplasm metastasis in lung ([2D70](#)) since it is reported in the same part of the certificate as the kidney cancer and the kidney cancer is considered primary.

Example 5

- 1 (a) Liver tumour {[2F90.Y](#)} [2C12.02](#)
(b)
(c)
(d)
- 2 Lung tumour, probably secondary [2D70](#)

Code both tumours as malignant, since the certifier described one of the two as secondary, which is evidence of malignant behaviour. Code the liver tumour as primary, since the other malignant neoplasm on the certificate is described as secondary. The qualification 'probably' is ignored; see Section [2.21.1 Uncertain diagnosis](#).

Example 6

- 1 (a) Metastatic involvement of chest wall [2E0Y](#)
(b) Carcinoma in situ of breast {[2E65.Z](#)} [2C6Z](#)
(c)
(d)
- 2

Code the carcinoma in situ of breast as malignant because it is reported as the cause of secondary spread, and code as primary at Malignant neoplasms of breast, unspecified (2C6Z) as the other malignant neoplasm is described as secondary.

Example 7

- 1 (a) Secondary malignant neoplasm of lung and brain [2D70](#), [2D50](#)
(b) Polyp of stomach {[DA44.Z](#)} [2B72.Z](#)
(c)
(d)
- 2

Code the polyp of stomach as malignant as it is reported as the cause of secondary spread, and code as primary at Malignant neoplasms of stomach, unspecified (2B72.Z) as all other malignant neoplasms are described as secondary.

Example 8

- 1 (a) Brain cancer (glioma) [2A00.0Z](#)
(b)
(c)
(d)
- 2 Kidney cancer [2C90.Z](#)

Brain is on the list of common sites of metastases, but the histopathology glioma indicates that it is primary in brain. Code the brain cancer as primary at Gliomas of brain, unspecified (2A00.0Z). Kidney is not on the list of common sites so code as primary at Malignant neoplasms of kidney, except renal pelvis, unspecified (2C90.Z).

Example 9

- 1 (a) Bone cancer (osteosarcoma) [2B51.Z](#)
(b) Colon cancer
(c)
(d) [2B90.Z](#)
- 2

Bone is on the list of common sites of metastases, but the histopathology osteosarcoma indicates that it is primary in bone. Code the bone cancer as primary at Osteosarcoma of bone and articular cartilage of unspecified sites (2B51.Z). Colon is not on the list of common sites so code as primary at Malignant neoplasms of colon, unspecified (2B90.Z).

Example 10

1 (a) Brain cancer [2A00.5](#)

(b)

(c)

(d)

2

Brain is on the list of common sites of metastases, but it is the only malignant neoplasm mentioned on the certificate and is not described as metastatic. Code the brain cancer as primary at Primary neoplasm of brain of unknown or unspecified type (2A00.5).

Example 11

1 (a) Cancer of cervical lymph nodes [2D60.0](#)

(b)

(c)

(d)

2

Lymph nodes are on the list of common sites of metastases, and it is the only malignant neoplasm mentioned on the certificate, but it is not described as primary. Code the cancer of cervical lymph nodes as secondary at Malignant neoplasm metastasis in lymph nodes of head, face or neck (2D60.0).

Example 12

1 (a) Cancer primary in prostate [2C82.Z](#)

(b)

(c)

(d)

2

The cancer is described as primary in prostate. Code at Malignant neoplasms of prostate, unspecified (2C82.Z).

Malignant neoplasm reported as secondary

If the certifier describes a neoplasm as secondary, code as a secondary malignant neoplasm at Malignant neoplasm metastases (2D50-2E2Z). Use the coding tool to find the appropriate code.

Other indication of secondary malignant neoplasm

Code a malignant neoplasm not specified as primary or secondary as secondary if:

- the site is on the list of common sites of metastases:
 - *exception*: if there is only one malignant neoplasm mentioned and it is not specified as 'metastatic', then code the neoplasm as primary. *note*: this does not apply to lymph nodes, which are always coded as secondary, even if it is

the only reported neoplasm on the certificate, unless it is specified as primary.

- *exception*: code lung as primary if all other sites in the same part of the certificate (Part 1 or Part 2) are on the list of common sites of metastases. However, code lung as secondary if one of the common sites of metastases reported on the same part of the certificate is considered primary, from its histopathology or by being described as caused by a known risk factor for malignant neoplasms of this site.
- *exception*: code a malignant neoplasm on the list of common sites of metastases as primary, if all other malignant neoplasms on the certificate are specified as secondary or as metastases. This applies whether or not these other malignant neoplasms are on the list of common sites of metastases.
- *exception*: code a malignant neoplasm on the list of common sites of metastases as primary, if the histopathology is stated and is compatible with the site. (To determine if a stated histopathology is compatible with the site, refer to textbooks or other reliable sources).
- unspecified whether primary or secondary, and the certifier states that the cancer is primary in another site. This applies whether or not the site is on the list of common sites of metastases:
 - regardless of site, do not code a neoplasm as secondary if it is of a different histopathology from another neoplasm stated to be primary. See also Section [‘2.21.5.3 More than one primary malignant neoplasm’](#).
- unspecified whether malignant, in situ or benign, and it is reported as due to a malignant neoplasm:
 - *exception*: if durations are stated, do not code the unspecified neoplasm as secondary if it has a duration that is longer than the durations of the malignant neoplasm reported as the cause of the unspecified neoplasm.
- the histopathology indicates that the neoplasm cannot be primary of the stated site. In that case, use both the default code for a primary neoplasm of the histopathology involved and a code for a secondary malignant neoplasm of the stated site.

If all sites are on the list of common sites of metastases, then code all sites as secondary. It is recommended adding a code for unknown primary. Code to [2D4Z Unspecified malignant neoplasms of unspecified sites], if no histopathology is stated. If the histopathology is stated, then code to the ‘unspecified site’ code for the histopathology involved. - *exception*: If all sites are on the list of common sites of metastases but one of them is lung, then code lung as primary.

If the certificate states that the primary site was unknown, then code all neoplasm sites mentioned on the certificate as secondary. (See also Section [2.21.5.5 Primary site unknown](#)).

Do not use order of entry alone to determine whether a neoplasm specified as malignant is primary or secondary. Code a malignant neoplasm reported as due to another malignant neoplasm as secondary only if it is described as secondary, metastatic spread or similar, or if it is on the list of common sites of metastases.

Do not confuse 'primary' with 'primary in'. While 'primary in' identifies one of several malignant tumours of the same or unspecified histopathology as the primary tumour, 'primary' simply means that the malignant neoplasm was not secondary. It does not necessarily mean that all other malignant neoplasms mentioned on the certificate were secondary.

Example 1

- 1 (a) Carcinoma of adrenal glands {[2D11.Z](#)} [2E07](#)
(b)
(c)
(d)
- 2 Primary in kidney [2C90.Z](#)

The malignant neoplasm of adrenal glands is considered secondary since the certificate states that the cancer was primary in kidney. Code the adrenal carcinoma as secondary at Malignant neoplasm metastasis in adrenal gland (2E07) and the primary in kidney as primary at Malignant neoplasms of kidney, except renal pelvis, unspecified (2C90.Z).

Example 2

- 1 (a) Prostate cancer {[2C82.Z](#)} [2E06](#)
(b) Primary site unknown
(c)
(d)
- 2

The primary site is described as unknown. Code to Unspecified malignant neoplasms of unspecified sites (2D4Z). Code prostate cancer as secondary at Malignant neoplasm metastasis in male genital organs (2E06) since the primary malignant neoplasm clearly was in another site.

Example 3

- 1 (a) Brain tumour {[2A00.5](#)} [2D50](#)
(b) Breast cancer [2C6Z](#)
(c)
(d)
- 2

Code the brain tumour as malignant, since it is reported as due to a malignant neoplasm breast cancer. Also, code as secondary at 2D50, since it is on the list of common sites of metastases. Code the breast cancer as Malignant neoplasms of breast, unspecified (2C6Z).

Example 4

- 1 (a) Brain tumour {[2A00.5](#)} [2D50](#)
(b) Lung cancer [2C25.Z](#)
(c)
(d)
- 2

Code the brain tumour as malignant, since it is reported as due to a malignant neoplasm lung cancer. Also code as secondary at Malignant neoplasm metastasis in brain (2D50), since it is on the list of common

sites of metastases and reported together with lung cancer. Code the lung cancer as primary at Malignant neoplasms of bronchus or lung, unspecified (2C25.Z), since the only other reported neoplasm is on the list of common sites of metastases.

Example 5

- 1 (a) Cancer growth in liver {[2C12.02](#)} and lymph nodes [2D80.0](#), [2D6Z](#)
 (b)
 (c)
 (d)
- 2 Malignant neoplasm of stomach [2B72.Z](#)

Code the cancer growth in liver and lymph nodes as secondary, at Malignant neoplasm metastasis in liver (2D80.0) and at Metastatic malignant neoplasm to unspecified lymph node (2D6Z), respectively, since they are both on the list of common sites of metastases. Also code the stomach as primary at Malignant neoplasms of stomach, unspecified (2B72.Z).

Example

6

- 1 (a) Cancer of lung, pleura {[2C26.Z](#)} and chest wall [2C25.Z](#), [2D72](#),
 {[2D4Z](#)} [2E0Y](#)
 (b)
 (c)
 (d)
- 2

Code the cancer of lung as primary at Malignant neoplasms of bronchus or lung, unspecified (2C25.Z), since the other sites mentioned on the certificate, pleura and chest wall, are on the list of common sites of metastases. Code cancer of pleura and chest wall as secondary at 2D72 and malignant neoplasm Malignant neoplasm metastasis in other specified sites (2E0Y), respectively.

Example 7

- 1 (a) Mesothelioma of pleura and lymph nodes [2C26.0](#), [2D60.Z](#)
 (b)
 (c)
 (d)
- 2

Pleura is on the list of common sites of metastases but the histopathology mesothelioma indicates that it is primary in pleura. Code as primary at Mesothelioma of pleura (2C26.0). Code the malignant neoplasm of lymph nodes as secondary at [2D60.Z](#), since lymph nodes is on the list of common sites of metastases.

Example 8

- 1 (a) Cancer of bladder [2C94.Z](#)
- (b) Cancer of kidney [2C90.Z](#)
- (c)
- (d)

2

Bladder and kidney are not on the list of common sites of metastases, and neither is described as primary, so code both as primary at Malignant neoplasms of bladder, unspecified (2C94.Z) and [2C90.Z](#) *Malignant neoplasms of kidney, except renal pelvis, unspecified*.

Example 9

- 1 (a) Osteosarcoma of sacrum [2B51.2](#)
- (b) Clear cell cancer of kidney [2C90.Y](#)
- (c)
- (d)

2

Code both malignant neoplasms as primary. Bone is on the list of common sites of metastases but the histopathology osteosarcoma indicates that it is primary in sacrum. Code as a primary at Osteosarcoma of bone or articular cartilage of pelvis (2B51.2). Also, it is of different histopathology than clear cell cancer of kidney.

Example 10

- 1 (a) Osteosarcoma of lung [2B51.Z](#), [2D70](#)
- (b)
- (c)
- (d)

2

The histopathology (osteosarcoma) indicates a primary neoplasm of bone, and the reported site (lung) is incompatible with the histopathology. Code as primary at Osteosarcoma of bone and articular cartilage of unspecified sites (2B51.Z), and add a code for Malignant neoplasm metastasis in lung (2D70).

2.21.5.3 More than one primary malignant neoplasm

If more than one primary malignant neoplasm is reported on the same certificate, code each primary malignant neoplasm as primary. Indications of more than one primary malignant neoplasms are:

- different histopathologies;
- a site-specific histopathology reported with a malignant neoplasm of another site that is not on the list of common sites of metastases;
- the sites are not on the list of common sites of metastases:

If one histopathology term is more specific and is an example of a more general histopathological term also reported on the certificate, then consider the two as referring to same neoplasm.

Do not consider 'cancer' a histopathologic term, but as synonym of 'malignant neoplasm'. On the contrary, 'carcinoma' is a histopathological term, describing a malignant tumour of epithelial origin.

Example 1

- 1 (a) Transitional cell carcinoma of bladder [2C94.2](#)
(b)
(c)
(d)
- 2 Osteosarcoma, primary in knee [2B51.1](#)

Bladder on line (a) is not on the list of common sites of metastases. The malignant neoplasm reported in Part 2 is specified as primary in knee but since the two neoplasms are of different histopathology, code both as primary at Urothelial carcinoma of bladder (2C94.2) and at Osteosarcoma of bone or articular cartilage of limbs (2B51.1), respectively.

Example 2

- 1 (a) Hepatoma [2C12.02](#)
(b) Cancer of breast [2C6Z](#)
(c)
(d)
- 2

Code hepatoma as primary at Hepatocellular carcinoma of liver (2C12.02), since the histopathology 'hepatoma' indicates a primary malignant neoplasm of liver. Code breast cancer as primary at Malignant neoplasms of breast, unspecified (2C6Z), since breast is not on the list of common sites of metastases.

Example 3

- 1 (a) Glioblastoma of brain [2A00.00](#)
(b) Cancer of breast [2C6Z](#)
(c)
(d)
- 2

Brain is on the list of common sites of metastases, but the histopathology glioblastoma indicates that it is primary in the central nervous system, usually in brain. Code glioblastoma of brain as primary at Glioblastoma of brain (2A00.00). Also code the breast cancer as primary at Malignant neoplasms of breast, unspecified (2C6Z), since breast is not on the list of common sites of metastases.

2.21.5.4 Site not clearly indicated

If a malignant neoplasm is described as in the 'area' or 'region' of a site, or if the site is prefixed by 'peri', 'para', 'pre', 'supra', 'infra' or similar expressions, then first check whether this compound term is included in the coding tool.

If the compound term is not in the coding tool, then code to the appropriate histopathology of the ill-defined site unspecified. Other specified malignant neoplasms of ill-defined or unspecified primary sites ([2D4Y](#)) is used for histopathologies specified but not classifiable elsewhere.

If neither of these apply, or the histopathology is not stated, then code to [2D42](#) *Malignant neoplasms of ill-defined sites*.

When the site of a primary malignant neoplasm is not specified, do not make any assumption of the primary site from the location of other reported conditions such as perforation, obstruction or haemorrhage. These conditions may arise in sites unrelated to the neoplasm. For example, intestinal obstruction may be caused by the spread of a malignant neoplasm of ovary.

Example 1

- 1 (a) Leiomyosarcoma in the region of the pancreas [2B58.Z](#)
- (b)
- (c)
- (d)
- 2

Code as Leiomyosarcoma, unspecified primary site (2B58.Z).

Example 2

- 1 (a) Cancer in the lung area [2C29.Z](#)
- (b)
- (c)
- (d)
- 2

Code as Malignant neoplasms of other or ill-defined sites in the respiratory system or intrathoracic organs, unspecified (2C29.Z)

Example 3

- 1 (a) Obstruction of intestines [DB30.Z](#)
- (b) Carcinoma [2D41](#)
- (c)
- (d)
- 2

Code the carcinoma as [2D41](#) Unspecified carcinoma of unspecified site (2D41)

2.21.5.5 Primary site unknown

If the certificate states that the primary site is unknown and does not mention a possible primary site or a specific histopathology, code to ([2D44](#)). If the primary site is unknown but a specific histopathology is stated code to the category for unspecified site for the histopathological type involved. For example, code adenocarcinoma to ([2D40](#)), and osteosarcoma to ([2B51.Z Osteosarcoma of bone and articular cartilage of unspecified sites]).

If the certificate mentions a probable or possible primary site, disregard the expression indicating doubt and code to that site. See also Section [2.21.1 Uncertain diagnosis](#).

If the certificate mentions several possible primary sites, select a code according to the instructions in Section [2.21.1.2 One condition, either one site or another](#) above.

Example 1

- 1 (a) Secondary carcinoma of liver [2D80.0](#)
 - (b) Primary site unknown [2D44](#)
 - (c)
 - (d)
- 2

The certificate states that the primary site is unknown. For line 1(b), code as [2D44](#).

Example 2

- 1 (a) Generalised metastases [2E2Z](#)
 - (b) Melanoma [2C30.Z](#)
 - (c) Primary site unknown [2D44](#)
 - (d)
- 2

The certificate states that the primary site is unknown, code to [2D44](#), Malignant neoplasm, primary site unknown, so stated. Code as [2C30.Z](#).

Example

3

- 1 (a) Secondary carcinoma of liver [2D80.0](#)
 - (b) Primary site unknown, [2B72.Z](#) *Malignant neoplasms of stomach, unspecified*
 - (c)
 - (d)
- 2

The certificate states that the primary site is unknown, but it also mentions stomach as a possible primary site. Ignore primary site unknown, 'possibly' and code line 1(b) as Malignant neoplasms of stomach, unspecified [2B72.Z](#) *Malignant neoplasms of stomach, unspecified*.

Example

4

- 1 (a) Secondary carcinoma of liver [2D80.0](#)
 - (b) Primary site unknown, probably stomach {[2B72.Z](#)} or colon [2C11.Z](#) {[2B90.Z](#)}
 - (c)
 - (d)
- 2

The certificate states that the primary site is unknown, but it also mentions stomach or colon as a possible primary site. Code line 1(b) as Malignant neoplasms of other or ill-defined digestive organs, unspecified (2C11.Z).

2.21.5.6 'Metastatic' cancer

Note: The expression 'metastatic' often poses a problem in the English language. Countries using languages other than English should translate only as much as needed of this section.

Neoplasms qualified as metastatic are always malignant. The adjective 'metastatic' can be used in two ways; mostly describing a primary tumour that has spread = metastasised to another site or organ (e.g. metastatic breast cancer), and sometimes describing the secondary site or location where the primary tumour has spread = metastasised (e.g. metastatic cancer in liver).

In the ICD-11 Foundation the adjective 'metastatic' followed by a site/organ (without a preposition) is always used in the meaning of a primary neoplasm of that particular site/organ that has spread into another site.

For multiple cause mortality coding, always follow the instructions in this section. This applies even if the coding tool indicates an ICD code for a 'metastatic' neoplasm or 'metastatic' disease other than the code you would arrive at by following these instructions. For example, the search might lead to a code in the section for 'malignant neoplasm metastases', but the multiple cause coding instructions might tell you to code the neoplasm as primary. If so, follow the instructions and code the neoplasm as primary.

Malignant neoplasm 'metastatic from' a specified site

If a malignant neoplasm is described as 'metastatic from' a specified site, or if a 'due to' relationship implies a spread from a specified site, code to primary of this site. This also applies to sites on the list of common sites of metastases. See Section [2.21.5.2](#) for the blocks used for primary malignant neoplasms.

Malignant neoplasm 'metastatic to' a specified site

If a malignant neoplasm is described as 'metastatic to' a specified site, or if a 'due to' relationship implies a spread to a specified site, code to secondary of this site, whether the site is on the list of common sites of metastases or not. Use a code in [2D50-2E2Z](#) 'Malignant neoplasm metastases' for this secondary site. However, if a histopathology is reported, code to the 'unspecified site' subcategory of that histopathological type.

Malignant neoplasm metastatic of site A to site B

A malignant neoplasm described as metastatic of site A to site B should be interpreted as primary of site A and secondary of site B.

'Metastatic' neoplasm of a specific histopathology

If the certificate reports a malignant neoplasm specified as 'metastatic' of a histopathological type classifiable to a cancer category that mentions a specific histopathology only, and the site reported is consistent with the histopathological type, then code to a primary malignant neoplasm of the specified histopathological type. Use the appropriate site subcategory for the specified histopathological type or site.

If the 'metastatic' cancer reported on the certificate and the site are not consistent with the histopathological type, then code to a secondary malignant neoplasm of the specified site.

Also add a code for a primary malignant neoplasm of unspecified site for the stated histopathological type.

When applying the remaining instructions on 'metastatic', do not change codes in [2A00-2A0Z](#) *Neoplasms of central nervous system or related structures*, [2B50-2B5Z](#) *Malignant mesenchymal neoplasms*, [2C30-2C3Z](#) *Malignant neoplasms of skin*, [2C40-2C4Z](#) *Malignant neoplasms of peripheral nerves or autonomic nervous system*, or assigned according to the instructions in this subsection 'Metastatic' neoplasm of a specific histopathology, to codes for secondary malignant neoplasms (2D50-2E2Z).

Example 1

- 1 (a) Osteosarcoma of sacrum, metastatic [2B51.2](#)
 - (b)
 - (c)
 - (d)
- 2

The site sacrum is consistent with a primary cancer of bone. Code as Osteosarcoma of bone or articular cartilage of pelvis (2B51.2).

'Metastatic' malignant neoplasm on the list of common sites of metastases

If the certificate mentions a single malignant neoplasm, and it is on the list of common sites of metastases and is specified as 'metastatic', then code the neoplasm as secondary, even if no other neoplasm is mentioned on the certificate. Also add a code for unspecified primary malignant neoplasm ([2D4Z](#)).

- *exception:* Code a neoplasm, even if described as 'metastatic', of a site on the list of common sites of metastases as primary when it is reported as due to a condition that increases the risk of a malignant neoplasm of that site or tissue.
- *exception:* If the only malignant neoplasm mentioned on the certificate is 'metastatic' neoplasm of lung, code to Malignant neoplasms of bronchus or lung, unspecified (2C25.Z). If another malignant neoplasm is mentioned that is not on the list of common sites of metastases, then code a 'metastatic' malignant neoplasm of lung as Malignant neoplasm metastasis in lung (2D70). This applies whether or not lung is mentioned in the same part of the certificate as the other malignant neoplasm.
- *exception:* For 'metastatic' neoplasms of a specified histopathology and on the list of common sites of metastases, see **'Metastatic neoplasm of a specific histopathology'** above.

Note that a malignant neoplasm of a site on the list of common sites of metastases is coded as primary if it is the only site mentioned and it is not described as 'metastatic'. See also **'Other indication of primary malignant neoplasm'** above.